The Repair Work

A Rupture Repair Protocol

For the Betrayed Partner

KekeTheLPC

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GA Integrated Therapeutic Perspectives LLC

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First Edition

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A Note on Privacy

The stories in this book are composites. Sarah and Mike, and every other person described in these pages, are not real individuals. They are built from patterns — from the shape of what happens again and again in the counseling room — with names, circumstances, and identifying details changed, combined, and invented to illustrate those patterns while protecting the privacy of the real people who lived them.

Any resemblance to a specific person is coincidental. Nothing in this book identifies, or is intended to identify, any actual client. What is true here is the pattern, not the person.

This book is educational. It is not therapy, and it is not a substitute for care from a licensed professional.

A Note from the Author

This book was written because the clinical language surrounding infidelity trauma often fails the people living it. Research articles use words like "post-infidelity stress disorder" and "amygdala hijack" and "attachment injury." These are real and useful terms. But they do not capture what it feels like to walk into your own kitchen and suddenly stop breathing.

This book does both. It meets you in the scene — on the porch, in the kitchen, in the bedroom, at the kitchen table — and then it tells you what is happening inside your body and why. It names what you are experiencing so you can stop wondering if you are going crazy.

You are not crazy. You are traumatized. And trauma, when understood and supported, can heal.

Read this book at your own pace. Sit with the chapters that hit hardest. Return to the ones that give you tools. Know that the author of these words is also a clinician, a woman, a friend, and a witness to what it means to survive the unsurvivable.

You are seen.

—KekeTheLPC

A Note on What This Book Is—and Is Not

Before you begin reading, I need to tell you something important about what this book is designed to do — and what it is not designed to do.

This book is for you if:

- Your partner is willing to engage in the work of repair.
- You are able to set boundaries in your relationship without fear of retaliation.
- You have the freedom to make choices about your future — including the choice to leave — without being threatened, controlled, or harmed.
- You are in a relationship where safety is possible, even if it doesn't feel safe right now.

This book is NOT for you if:

- Your partner has ever been physically violent, threatened you, or made you afraid for your safety.
- Your partner controls your access to money, resources, or people who support you.
- Your partner uses the affair to blame, shame, or control you.
- You are afraid to leave — not because you love your partner, but because you are afraid of what they will do.
- Your partner has isolated you from friends, family, or professional support.

• Your partner has refused to go to therapy or has used therapy to manipulate you.

If any of these apply, please put this book down. Contact the National Domestic Violence Hotline at 1-800-799-7233 or text START to 88788. Your safety is more important than this relationship.

Infidelity that occurs in the context of abuse is not the same as infidelity that occurs in a relationship where both partners can safely do the work. In abusive relationships, the advice in this book — about patience, watching, and giving your partner a chance — could put you at greater risk. The work of repair requires safety. Without safety, there is no repair — only survival.

If you are in an abusive relationship, your work is not to heal the relationship. Your work is to get safe.

This book assumes that your partner is willing to read the companion volume, go to therapy, sit in discomfort, and change their behavior. That is a specific set of conditions. If those conditions are not present — if your partner is not willing, if they are defensive, if they are using the affair as a weapon — this book cannot help you. That is not a failure of the book. It is a failure of your partner.

CHAPTER ONE

The Ambush

The Scene

Sarah comes home early. Mike is in the kitchen making pasta. The song is playing — the one from D-Day.

She freezes in the doorway. Keys still in her hand. Heart already pounding.

Sarah: Mike. Turn it off. Please.

Mike: What? The music?

Sarah: That song. It's the song. From the porch. From the day I—

Mike: (realizing, reaching for his phone) Shit. I'm sorry. I didn't even notice.

He turns it off. Silence.

Sarah: (still frozen, voice tight) I can't breathe.

Mike: (putting down the spatula, walking toward her) Hey. It's okay. It's off. You're okay. Just breathe.

Sarah: I was fine. I was having a good day. And then I walked in and it was just—I was back there. On the porch. Hearing your voice. Hearing you say—

Mike: I know. I know. I'm sorry.

He reaches for her. She stands stiff in his arms. Her heart is still pounding against his chest.

Mike: (quietly) I've got you.

Sarah: (pulling back, wiping her face) I hate this. I hate that a stupid song can do this to me. I hate that I can't just walk into my own kitchen without falling apart.

Mike: It's not stupid. It's trauma.

Sarah: I don't care what it is. I just want to stop feeling like I'm dying every time something reminds me of what you did.

What Just Happened?

Sarah was ambushed.

Not by Mike. Not by the song. She was ambushed by her own nervous system. In the span of a single breath, she was ripped out of the present and thrown back into the worst moment of her life. Her body responded as if the affair was happening again — because, to her brain, it was happening again.

This is not an overreaction. This is not weakness. This is not a failure to "get over it."

This is biology.

The Physiology of the Ambush

What Sarah just experienced is documented in research on infidelity-based trauma. Victims of romantic betrayal report persistent somatic symptoms: difficulty breathing, bodily tremors, extreme nervousness, and a racing heart when recalling the betrayal. They experience insomnia, weight loss, difficulty concentrating, and a loss of appetite and libido.

The moment Sarah heard the song, her brain responded before her mind could.

The amygdala — the brain's alarm system — detected a threat. It triggered a cascade of stress hormones. Adrenaline flooded her system. Cortisol surged. Her heart raced. Her vision narrowed. Her body prepared to fight or flee.

Her prefrontal cortex — the rational, thinking part of her brain — went offline.

This is why she could not "think" her way out of the panic. Research shows that in betrayal trauma, communication between the prefrontal cortex and the amygdala breaks down. Victims may intellectually understand that the threat is gone, but their body still reacts as if it is happening again.

Her hippocampus — the memory organizer — failed to time-stamp the memory.

In trauma, the hippocampus may not fully integrate the betrayal into conscious memory. This is why Sarah was not just remembering D-Day — she was reliving it. The past collapsed into the present. Her brain could not distinguish between the memory and the current moment.

This is an ambush.

A trigger activates the trauma memory. Without new information to change the brain's response, fear and distrust grow stronger. The brain becomes wired for reactivation, even without new betrayal.

The Symptoms Are Real

What Sarah is experiencing is not unusual. It is documented in research.

93% of betrayed partners exceed the cut-off score for probable post-traumatic stress disorder (PTSD).

71.6% meet or exceed clinically significant depression symptoms.

73.9% meet or exceed clinically significant trauma-like symptoms.

66.4% meet or exceed clinically significant anxiety symptoms.

Women who experience infidelity are six times more likely to be diagnosed with a major depressive episode than those who have not experienced it.

Betrayed partners report: persistent sadness, crying spells, difficulty concentrating, insomnia, nightmares, hypervigilance, avoidance, irritability, emotional volatility, loss of interest, rumination, lack of appetite, and loss of libido.

They experience flashbacks, triggers, hypervigilance, avoidant behavior, and manifestations researchers have named Post-Infidelity Stress Disorder.

They feel rage — even after progress. They feel isolated and betrayed and have a sense of loss. They feel confused and desperate after experiencing such a breach of trust.

The Physical Toll

Betrayal does not only affect the mind. It affects the body.

Research shows that those who report romantic betrayal by a partner are more likely to also report chronic health issues. The effects include higher risks of stress-related medical conditions, particularly endocrine and autoimmune disorders.

One betrayed partner developed blood clots and psoriasis after discovering her husband's affair. Her doctors could only say that her central nervous system was so shaken up because she went through a traumatic event.

What This Means for You

If you have felt your body react to a trigger — your heart racing, your stomach dropping, your vision narrowing, your mind spinning — you are not crazy.

You are experiencing a normal, predictable, physiological response to trauma. Your body is doing exactly what it was designed to do: protect you from danger. It just does not know

that the danger is in the past.

You are not weak. You are not broken. You are not failing at healing.

Your triggers are not a setback. They are a message from your body that something still needs to be healed. They are a sign that your nervous system is trying to protect you — even if the danger is over.

The Truth You Need to Hear

You are not alone.

Millions of people have walked this path before you. They have felt what you are feeling. They have doubted themselves. They have doubted their partners. They have doubted everything they thought they knew.

You are not broken.

Healing from betrayal requires addressing both emotional and physiological dimensions — retraining the body to experience safety again while rebuilding trust in your perceptions. The brain is plastic. It can heal. With care, safety, and support, the brain can return to balance.

But it cannot be rushed. It cannot be shamed. It cannot be forced.

You are allowed to need time. You are allowed to set boundaries. You are allowed to say: "I'm not ready. I don't know when I'll be ready. I need time."

You deserve to feel safe. Not eventually. Not after you've been patient enough. Now.

You deserve to be held in the dark.

You deserve to be seen.

End of Chapter One

CHAPTER TWO

Self-Regulation — Stabilizing When the Ground Shifts

The Scene Continues

Sarah is standing in the kitchen. Mike just turned off the song. Her heart is still pounding. Her hands are shaking. Her vision is still narrowed. She can't think clearly. She can't speak. She can't breathe.

Sarah: (struggling) I can't—I can't breathe.

Mike: (reaching for her) Hey. I've got you. Just breathe.

Sarah: (pulling back) No. No. I can't—I need—I need air. I need to move.

She walks out the back door. She stands on the porch. She puts her hands on the railing. She feels the wood. Rough. Solid. Real.

Sarah: (quietly, to herself) Five things I can see. The fence. The tree. The bird. The sky. My hands.

She looks at her hands. They're still shaking.

Sarah: Four things I can touch. Wood. My jacket. My hair. My wedding ring.

She touches the ring. She almost takes it off. She doesn't.

Sarah: Three things I can hear. The wind. A car. My breathing.

She takes a breath. Slow. In for four. Hold for four. Out for eight.

Sarah: Two things I can smell. The grass. The rain coming.

Sarah: One thing I can taste. Salt. From crying.

She takes another breath. Her heart is still pounding, but it's slowing. Mike comes to the door. He doesn't walk out. He just stands there, watching. Waiting.

Mike: Are you okay?

Sarah: (still breathing, still grounding) I'm getting there.

Mike: Do you want me to come out? Or do you want space?

Sarah: (thinking) Just stay there. I need—I need a minute.

Mike: Okay. Take your time.

He stays in the doorway. He doesn't push. He doesn't rush. He just waits.

Sarah: (finally) Okay. I'm okay.

They go inside. Mike makes tea. He doesn't ask questions. He just hands her a mug and sits next to her on the couch.

Sarah: (quietly) Thank you for not rushing me. For not sighing.

Mike: I'm sorry I did that. Earlier. I won't do it again.

Sarah: (nodding) I know.

What Sarah Just Did

Sarah just saved herself. Not because Mike was there. But because she used the tools she'd been learning — the tools that take a person from drowning to treading water.

She did not "get over it." She did not suppress her feelings. She regulated herself.

Regulation is not the same as healing. It is not the same as forgiveness. It is the ability to bring your nervous system back from the brink — to pull yourself out of the flood and back onto solid ground.

This is the single most important skill you can learn.

The Physiology of Regulation

When the trigger hit, Sarah's nervous system went into high alert. What she did to come back:

- Movement. She walked out of the kitchen. Movement metabolizes adrenaline. It tells the body: we are not in danger.
- Grounding. She used the 5-4-3-2-1 method. This forced her prefrontal cortex to re-engage. She could not be in a trauma response while counting sensory details. The two are physiologically incompatible.
- Breathing. In for four, hold for four, exhale for eight. The long exhale activates the vagus nerve — the off-switch for the fight-or-flight response.
- Touch. She felt the wood, her hair, her ring. Physical sensation brings the body into the present.
- Space. She asked for space and Mike respected it. This is a boundary — and boundaries are self-regulation.

The Regulation Toolkit

Here are the tools Sarah used. They are simple. They are not easy. They require practice. But they work.

1. The 5-4-3-2-1 Grounding Method

> Name: 5 things you can see · 4 things you can touch · 3 things you can hear · 2 things you can smell · 1 thing you can taste.

Why it works: This forces your prefrontal cortex to re-engage. You cannot be in a full amygdala hijack while counting and naming sensory details. The two states are mutually exclusive.

2. The Vagus Nerve Breath

Inhale through your nose for 4 counts · Hold for 4 counts · Exhale through your mouth for 8 counts.

The long exhale activates the vagus nerve. Do this 3–5 times. You will feel your heart rate slow.

3. Movement

Adrenaline is designed for physical action. When you don't fight or flee, that adrenaline stays in your system. Move. Walk. Shake out your hands. Go for a run. Movement metabolizes adrenaline.

4. Temperature Change

Cold water on your face. An ice cube in your hand. A cold shower. Temperature change activates the mammalian dive reflex — a physiological response that slows your heart rate.

5. Physical Touch

Hold something. Press your feet into the floor. Put your hand on your chest. Touch releases oxytocin, which counteracts cortisol.

6. The Verbal Anchor

"I am being triggered. This is a trigger. I am not dying. I am not back there. I am in the present. The affair is over. I am safe. I am okay. This will pass. It always passes. I am not alone."

Your brain listens to what you say. When you say "I am safe" out loud, your amygdala hears it.

The Truth You Need to Hear

You are not failing at healing because you still get triggered. You are not weak because you need to regulate. You are doing

the hardest work of your life.

Regulation is not a sign of failure. It is a sign of survival.

End of Chapter Two

CHAPTER THREE

The Intimacy Interruption — When Sex Becomes Complicated

The Scene

It's been four months since D-Day. Sarah and Mike are in couples counseling. But there's one thing they haven't worked on. The bedroom.

Tonight, Mike tries.

Mike: (sitting next to her, reaching for her hand) Sarah. Can we talk?

Sarah: (pulling her hand back) About what?

Mike: Us. The bedroom. I feel like you're avoiding me.

Sarah: (quietly) I can't. I can't just turn it on. I can't just pretend everything is fine and climb into bed with you like nothing happened.

Mike: I'm not asking you to pretend. I'm asking you to be with me. It's been months.

Sarah: (voice rising) You think I don't want to want you? Every time I think about being with you, I see her. I feel like I'm dying.

Mike: (defensive) That's not fair. I ended it. I chose you.

Sarah: I know you're trying. But that doesn't make the images go away.

Mike: You are enough. You've always been enough.

Sarah: (crying) Then why wasn't I enough for you? Why did you need her?

Mike is silent. He doesn't have an answer.

Sarah: (crying) I look at myself in the mirror and I see someone who wasn't enough. And then I think about being with you — being naked, being vulnerable, trusting you again — and I can't breathe. Because the person who hurt me the most is the same person who's supposed to make me feel safe.

Mike: What do you need from me?

Sarah: I don't know. I just know I can't. Not yet. Not until I feel like I'm not just the consolation prize.

Mike: (quietly) You're not the consolation prize. You were never the consolation prize.

Sarah: Then prove it. Show me. Not with words. With time. With patience. With consistency. Show me that I'm not going to be hurt again.

Mike: (nodding) I can do that.

What Just Happened?

Sarah is not withholding sex to punish Mike. She is protecting herself. Her body is telling her — loudly and clearly — that she is not safe. That vulnerability equals danger.

This is not a choice. This is trauma.

Infidelity interrupts sex. Not because the betrayed partner is "cold" or "withholding" or "using sex as a weapon." But because

the connection between sex, trust, safety, and vulnerability has been severed. The affair didn't just break trust. It broke the sexual bond.

The Physiology of Sexual Trauma After Infidelity

When Sarah thinks about being intimate with Mike, her amygdala fires. Her nervous system goes into high alert. In her body: adrenaline surges, cortisol spikes, her pelvic floor tightens as a protective response, she may feel numb or dissociated — survival mechanisms all. In her mind: intrusive images, relentless comparison, shame, fear of vulnerability.

What the Research Says

Research shows that infidelity has a devastating impact on sexuality and intimacy. Betrayed partners often experience significant loss of sexual desire. They may find themselves avoiding their partner's sexual advances. When they do engage in sex, they may feel angry or disgusted during or after the encounter. Many report feeling pressured to engage in "reclamation sex" that feels more like a performance than an intimate act.

Post-infidelity sexuality is a process, not an event. It requires safety, patience, and transparency. It requires the unfaithful partner to be willing to wait, to offer non-sexual intimacy, and to prove — with consistency — that they see their partner as more than just a body.

What You Need to Know

You are not broken. You are not cold. You are not withholding. You are protecting yourself. And you are allowed to say no. You are allowed to take time.

What you need to feel safe:

• Patience. Real patience. The kind that doesn't keep score. The kind that says: "I'll wait. For as long as it takes. No strings attached."

• Transparency. No secrets. No guarded phones. No suspicious behavior.

• Consistency. Trust is rebuilt in the thousands of small moments that follow the betrayal.

• Non-sexual intimacy. Touch that doesn't lead anywhere. Touch that says: I want you, not just your body.

• Time. Trauma does not heal on a schedule.

You deserve to be held. You deserve to be seen. You deserve to be safe.

End of Chapter Three

CHAPTER FOUR

Boundaries — The Architecture of Safety

The Scene

It's been six months since D-Day. Something has shifted. Sarah is starting to see things more clearly. She's starting to understand what she needs — and what she will no longer tolerate.

Sarah: I need to talk to you about something. And I need you to just listen. Don't defend. Don't explain. Just listen.

Mike: (nodding) Okay.

Sarah: First. The phone. I need you to leave it face-up on the counter when you're home. If you need to use it, tell me. Not because I'm checking up on you. But because I need to see — with my own eyes — that you have nothing to hide.

Mike: (nodding) I can do that.

Sarah: Second. No more late nights without telling me where you are. If you're running late, text me. I need to know you're thinking about me.

Mike: I can do that.

Sarah: Third. No more defensiveness. When I'm triggered, I need you to just listen. Don't defend. Just say: "I hear you. I'm sorry. I'll do better."

Mike: (swallowing) I can do that.

Sarah: Fourth. I need you to be patient with my healing. I don't know how long this is going to take. And I need you to be okay with that. To say: "I'll wait. For as long as it takes. No strings attached."

Mike: I can do that.

Sarah: (voice breaking) I need you to mean it. Not just say it. Because if you can't — if you're just waiting for me to get over it — I need to know. Because I can't do this again.

Sarah: (after a moment) Fifth. I need you to go to individual counseling. Figure out why you did this. Do the work — not for me, but for you.

Mike: (quietly) I'll go. I'll find someone this week.

Sarah: Sixth. I need you to be honest with me — about everything. Because trust isn't just about the big things. It's about the thousands of small moments.

Sarah: And seventh. I need you to understand that these boundaries are not punishment. They're about protecting me. They're about giving me a reason to feel safe.

Mike: (without hesitation) I can do all of it. Every single thing. Not because you're asking me to. But because I want to be the man you deserve.

Sarah: (crying) Okay.

Mike: Can I hug you?

Sarah: (nodding) Yeah.

He holds her. Not tightly. Not desperately. Just present. Just patient. Just there.

What Sarah Just Did

Sarah just set boundaries. Not to punish Mike. Not to control him. To protect herself. To give her nervous system something it desperately needed: a reason to believe she was safe.

Boundaries are not walls. They are not punishments. They are the framework that makes trust possible. Without boundaries, there is no trust.

What Boundaries Are

- Boundaries are not punishment. They are about protection.
- Boundaries are not walls. They define what is acceptable and what is not.
- Boundaries are not selfish. They acknowledge that your needs matter.
- Boundaries are not permanent. As trust is rebuilt, boundaries can relax. But they must be earned.
- Boundaries are not a sign of weakness. They are a sign that you know what you need.

What Boundaries Look Like

Around transparency: phone accessibility, honest communication, knowing where your partner is.

Around defensiveness: listening without defending, saying "I hear you" instead of "you're overreacting."

Around time: "I don't know how long this will take. I need you to be okay with that."

Around honesty: complete truth about even small things.

Around personal work: individual therapy, real self-examination.

Around intimacy: no pressure, non-sexual touch first, time to rebuild.

The Truth You Need to Hear

Your boundaries are not optional. They are not suggestions. They are the conditions under which you can heal.

You deserve to feel safe. Not eventually. Not after you've given enough chances. Now.

End of Chapter Four

CHAPTER FIVE

What Healing Actually Looks Like

The Scene

It's been eight months since D-Day. Sarah is sitting on the couch. Mike is reading on the other end. His phone is on the counter, face-up.

Sarah: (quietly) Mike?

Mike: (looking up) Yeah?

Sarah: I just realized something. I haven't thought about her all day. I mean, I thought about her. But I didn't really think about her. I didn't feel like I was dying. I just... I just thought about her and it didn't hurt the same way.

Mike: (setting down his book) That's good. That's really good.

Sarah: Is it? Or am I just... forgetting? Pretending it didn't happen?

Mike: I don't think you could pretend if you tried.

Sarah: (small smile) No. I couldn't.

Sarah: (thinking) Like I'm not drowning anymore. Like I'm still in the water, but I can see the shore.

Mike: (reaching for her hand) I'm sorry it took so long. I'm sorry I made it harder.

Sarah: (not pulling away) You did make it harder. But you're not making it harder anymore. That counts for something.

Mike: It counts for everything.

Sarah: (quietly) I'm not saying I forgive you. I don't know if I ever will. But I'm not drowning anymore. That's something.

What Healing Looks Like

Healing does not look like what you think it looks like. It is not linear. It is not neat. It is messy. It is full of setbacks. It looks like two steps forward and three steps back.

Healing does not mean forgetting.

It does not mean you never get triggered. It means those things happen less often. They hit less hard. They take less time to recover from.

Healing does not mean returning to who you were.

You will never be the person you were before the affair. That person is gone. You are becoming someone new — someone who knows what betrayal feels like, who knows she can survive the unthinkable.

The Stages of Healing

Healing is not a checklist. But there are stages — messy, overlapping, non-linear stages — that most people move through.

Stage 1: The Shattering

The discovery. The shock. The collapse of everything you thought was true. What healing looks like here: just surviving. Just getting through the day. Not making big decisions.

Stage 2: The Flood

The numbness wears off. The feelings come. All of them. At once. What healing looks like here: letting yourself feel. Finding safe outlets. Learning to regulate. Starting to set boundaries.

Stage 3: The Wrestling

You go back and forth. One day you want to stay. The next you want to leave. What healing looks like here: staying in the question. Watching your partner's behavior, not their words.

Stage 4: The Grieving

You are starting to accept what happened. Not forgive. Accept. A deep, quiet sadness. Less sharp than the flood, but heavier. What healing looks like here: allowing yourself to grieve. Imagining a new future.

Stage 5: The Rebuilding

You are no longer defined by the affair. It is part of your story, but not the whole story. What healing looks like here: rebuilding your sense of self, rebuilding trust if you choose to stay.

The Milestones

You will know you are healing when:

- You go a whole day without thinking about the affair.
- A trigger hits and you recover in minutes instead of hours.
- You laugh. Really laugh. Without it feeling like a betrayal of your pain.
- You look at yourself in the mirror and do not see someone who was replaced.
- You trust your own judgment again.
- You can imagine a future — with or without your partner.
- You feel something that looks like joy.

The Truth You Need to Hear

You are healing. Even when it doesn't feel like it.

Every day you get out of bed. Every day you feel the pain and don't run from it. Every day you breathe through a trigger. Every day you keep going.

That is healing.

End of Chapter Five

CHAPTER SIX

The Decision — How to Know If You Should Stay or Leave

The Scene

It's been a year since D-Day. Sarah is sitting at the kitchen table. Mike is at work. She has a journal open in front of her.

Today, she is trying to answer the question she has been living inside for twelve months.

I don't know if I can do this anymore. But I know I'm not the person I was before. I know I can survive anything. I know what I need. And I know what I will no longer tolerate.

The Questions You Need to Ask Yourself

Do not ask: "Do I still love him?" Love is not enough. Love is what kept you in the relationship. Love is not a reason to stay.

Do not ask: "Will I be okay without him?" You will. You have already survived the worst thing that could happen to you.

Ask yourself these instead:

1. Is my partner truly accountable?

Not just sorry. Not just trying. Actually accountable. Do they take full responsibility without excuses? Do they understand why they did it? Are they in individual counseling? Have they changed — not just their behavior, but their character?

If the answer to any of these is no, you have your answer.

2. Am I safe with my partner?

Not physically safe. Emotionally safe. Can I be vulnerable without fearing I will be hurt again? Do they respect my boundaries? Do they make me feel like my healing is an inconvenience?

If you do not feel safe, you cannot stay. Safety is not negotiable.

3. Is my partner patient?

Do they rush me? Ask when I'll be "over it"? Pressure me for intimacy? Or do they say "I'll wait, for as long as it takes" and actually mean it?

4. Is my partner transparent?

Not just saying they have nothing to hide. Actually transparent. Phone face-up. Honest about small things. No defensiveness.

5. Am I staying out of fear or love?

Fear of being alone. Fear of starting over. Fear of what others will think. Or: real love, grounded in reality, seeing genuine change?

If you are staying out of fear, you need to leave. Fear is not a foundation for a relationship. It is a prison.

The Signs That You Should Stay

• Your partner has taken full accountability — no excuses, no deflecting.

• Your partner is doing the work: individual therapy, sitting in shame, changing.

• Your partner is patient and transparent — proving it every day.

• You feel safe. Not always. But more often than not.

• You want to stay — not out of fear, but because you believe it is the right choice.

The Signs That You Should Leave

• Your partner is not accountable and not doing the work.

• Your partner rushes you or pressures you.

• You do not feel safe — still hypervigilant, still waiting for the next blow.

• You are staying out of fear, not love.

• You have lost yourself entirely.

The Truth You Need to Hear

There is no right answer. There is only the choice that is right for you — right now, with the information you have.

You are allowed to choose yourself.

You are not responsible for your partner's healing. If you leave, they will survive. Your job is to heal yourself. Your job is to protect yourself. Your job is to choose yourself.

Whatever you choose — stay or leave — you will survive. You deserve to feel safe. You deserve to be loved. Trust yourself.

End of Chapter Six

CHAPTER SEVEN

Your Anger Has a Job

The Scene

Sarah is in her car in the parking lot of the grocery store.

She has been there for eleven minutes. She knows because she is staring at the clock on the dashboard and has been unable to stop staring at it since she got in the car.

She ran into her neighbor inside. The neighbor asked about Mike — sweetly, casually, the way people ask when they have no idea. How is Mike? How is everything?

Sarah said fine. She smiled. She paid for her groceries. She walked to her car.

Now she is sitting here and her hands are shaking and there is a sound building inside her that she does not know what to do with.

She puts both hands on the steering wheel.

She holds on.

The sound comes out as a scream she immediately muffles with the back of her hand. Then another one. Then she is hitting the steering wheel — not hard, just repeatedly, just to have somewhere for her hands to go.

She stops. Looks at her hands. Breathes.

She is not having a breakdown. She is not losing her mind. She is not becoming someone she doesn't recognize.

She is angry.

And her anger has a job to do.

What Anger Actually Is

In clinical settings, anger after betrayal is sometimes pathologized — treated as a symptom to manage, a stage to move through, evidence of an inability to cope. This is wrong.

Anger after betrayal is not dysfunction. It is information.

Your anger is your nervous system's assessment of what happened to you. It says: This was wrong. I was wronged. What was done to me violated something real. I am not okay with this.

That assessment is accurate. You were wronged. What was done to you violated something real. You are not okay with this. Your anger knows.

Anger is not the opposite of healing. Anger is part of healing. The question is not how to get rid of it — but how to let it do its job.

Research on trauma and betrayal consistently shows that suppressing anger increases the duration and severity of trauma symptoms. Partners who were told to 'calm down,' 'be rational,' or 'stop bringing it up' consistently report longer recovery times and higher rates of unresolved grief. This is not a coincidence.

Anger needs somewhere to go. If it doesn't have permission to move through you constructively, it goes somewhere else: into your body (physical symptoms — headaches, insomnia, GI distress), into the relationship (constant low-grade conflict, cycles of withdrawal and explosion), or inward (depression, self-blame, shame).

None of those outcomes serve you. Letting anger do its job does.

What Anger Is Telling You

Your anger is not random. It has information embedded in it. If you listen to it — rather than suppress it or drown in it — you will find specific data about what was violated, what you need, and what matters most to you.

Betrayal anger typically carries several messages:

• My trust was violated. I trusted this person with the most intimate parts of my life, and they used that trust as cover for deception.

• My reality was stolen. I was making decisions about my life based on information I didn't know was false. I don't know which of my decisions were based on lies.

• My body was at risk. This is not discussed enough: sexual betrayal puts a partner's physical health at risk without their consent. That violation is real and legitimate.

• My time was taken. Every moment spent in this relationship — while the other person was lying — was a moment in which I was not able to make an informed choice about my own life.

• My dignity was dismissed. What was done to me required actively devaluing my experience, my feelings, and my reality. That devaluation is an injury.

When you know what your anger is about specifically, you can begin to address the actual wound — not just manage the emotion sitting on top of it.

The Difference Between Anger and Rage

There is an important distinction between anger and rage. Anger is information. Rage is what happens when anger has nowhere to go for too long — when it builds and pressurizes until it finds the nearest exit.

Rage is not useful in the same way anger is. Rage does not communicate clearly. It does not set limits — it obliterates them. It does not protect — it destroys.

If you find yourself in rage states — screaming things you don't mean, threatening things you won't do, unable to stop once started — this is a signal that your anger has been suppressed or dismissed for too long and has built to unsustainable pressure.

Rage states are not evidence that you are 'too much' or 'out of control.' They are evidence that you have been trying to contain an emotion that required processing, and the container failed. The solution is not more containment. It is finding a constructive channel.

What to Do With the Anger

Tools for Productive Anger Processing

Anger is energy. It needs to move. The following are evidence-based approaches to processing anger constructively — not suppressing it and not weaponizing it:

1. Physical discharge. Anger is a physiological state. The adrenaline and cortisol that flood your body during anger are designed to fuel action. When you can't take the action your body is preparing for, that energy has nowhere to go. Move your body: run, walk fast, swim hard, do push-ups until your arms give out. You are not 'working off' the anger — you are giving the physical energy somewhere to land.

2. Write without editing. Get a journal — physical paper is better for this. Set a timer for twenty minutes and write about what you are angry about without stopping, without editing, without worrying about grammar or fairness or whether you 'really mean it.' You may write things you would never say out loud. Write them. No one will read this. The point is to let it move through you onto the page.

3. Say it out loud in private. Some people find it useful to speak their anger aloud when alone — in the car, in the shower. Say the things you can't say to your partner. Say them at full volume if

you need to. You are not rehearsing a fight — you are processing an emotion. There is a difference.

4. Name it in session. Individual therapy is the place where anger can be fully expressed, witnessed, and worked with. Your therapist can help you understand what each specific strand of anger is about, and what it needs.

5. Bring it directly (when you are regulated). The most powerful use of anger is as communication — clear, specific, non-weaponized. When you are regulated enough to speak in full sentences, there is value in telling your partner specifically what you are angry about. Not to punish them. Not to 'win.' But because your anger contains information they need to understand the full scope of what they did.

What not to do with the anger:

• Do not suppress it for the sake of 'keeping the peace.' You will pay for that suppression in your body and your mental health.

• Do not perform it to make your partner feel worse. Anger that is deployed for maximum impact on the other person has left its own territory and entered manipulation. This will not serve you.

• Do not make major decisions from inside it. Do not file for divorce, move out, or issue ultimatums from inside a rage state. Let the acute physiology settle before you make choices that will have long-term consequences.

• Do not tell yourself it is 'not okay' to feel it. It is completely okay to feel it. It is appropriate to feel it. It is a rational response to an irrational injury.

You do not have to forgive your anger. You have to let it speak. Then listen to what it says.

CHAPTER EIGHT

Grief

The Scene

Sarah finds the box while looking for something else in the back of the closet.

It is a shoebox. She knows what is in it before she opens it — photos from Costa Rica, the trip they took three years ago for their anniversary. The trip where they danced badly in a bar until two in the morning and came back sunburned and laughing and fell asleep fully clothed on the hotel bed.

She opens it anyway.

She is looking at herself in the first photo — on a beach, squinting into the sun, grinning at whoever is behind the camera. At Mike. She is looking at Mike.

She does not recognize the person in this photo. Not because she looks different. Because she was different. She was someone who trusted without question. Someone for whom the ground was solid.

That person is gone.

She closes the box. She sits down on the closet floor.

She does not know what to call what she is feeling. It is not rage. It is not the sharp blade of the trigger. It is something much older and quieter and more total.

It is grief.

What You Are Actually Grieving

One of the least discussed aspects of betrayal recovery is grief. Not anger — anger gets a lot of airtime. Not fear. Grief.

Betrayal does not just break a relationship. It retroactively rewrites one. When you discover what happened, you do not only lose the future you planned — the one that included this person, this partnership, this shared life. You also lose the past you thought you had.

The vacation. The lazy Sunday mornings. The conversation you had last week about growing old together. All of it is now under question: Was any of it real? What was I actually in?

This is the specific cruelty of betrayal grief: it robs you of your own history. You cannot look back at the last several years of your life and see them clearly. You are standing on ground you thought was solid, discovering that it was a stage set.

You are grieving the relationship you thought you were in — which is different from the relationship that actually existed. And that grief is legitimate, even though the thing you're mourning was never exactly as you experienced it.

Ambiguous Loss

Grief after betrayal is a form of what psychologist Pauline Boss calls 'ambiguous loss' — loss that does not have a clean boundary. The person has not died. The relationship has not officially ended. The loss is real, but it does not have the clear social rituals and recognition that typically accompany grief.

No one brings you a casserole. No one sends a card. No one says 'I'm so sorry for your loss,' because to many people looking at your life from the outside, nothing is lost. You are still married. He is still there. What are you grieving?

This lack of social recognition makes betrayal grief harder to bear. You are moving through a mourning process without the

scaffolding that mourning typically provides. You may not even have permission from yourself to grieve — you may be telling yourself that grief is for people whose relationships end, not for people in the middle of trying to repair one.

But the grief is there regardless of what you tell yourself. And unacknowledged grief does not disappear. It goes underground, becomes depression, becomes numbness, becomes a flat affect that no amount of 'working on the relationship' seems to reach.

You need to grieve. Even if the relationship survives. Even if you decide to stay. The version of your relationship that you thought you had is gone. That deserves to be mourned.

What Grief Looks Like in Betrayal

Betrayal grief does not always look like crying. It sometimes looks like:

• An inability to feel joy — not because you are depressed exactly, but because pleasure requires a sense of safety that has been removed.

• Emotional flatness — days when you feel nothing, when you go through the motions without any feeling behind them.

• Nostalgia that hurts — memories of the 'before' that land with a quality of loss rather than warmth.

• Difficulty with the future — an inability to imagine ahead, to make plans, because the foundation on which you were building plans has shifted.

• A sense of having become a different person — because you have. The person you were before you knew is not exactly who you are now.

All of these are manifestations of grief. None of them are weakness. All of them deserve your compassion.

Grieving Forward

The goal of grief is not to stop feeling loss. It is to integrate the loss — to carry it in a way that allows you to also carry other things. To live alongside the grief rather than being swallowed by it.

Grief researcher William Worden identified four tasks of grieving that are useful here:

1. Accepting the reality of the loss. Not the affair — you know that happened. But accepting the specific reality of what was lost: the relationship as you understood it, the person you thought you knew, the security of the ground.

2. Working through the pain of grief. This means not avoiding the feelings. Not 'staying busy.' Sitting with the grief long enough to let it move through you rather than building behind a dam you have to maintain constantly.

3. Adjusting to a world in which the loss is real. Finding a way to live in the version of your life where this happened. Discovering who you are in this new terrain.

4. Finding an enduring connection to what was lost while embarking on a new life. You do not have to amputate the good parts of your shared history. You can carry them differently — understanding them in a more complete context, neither discarding nor enshrining them.

This is not a linear process. You will move in and out of these tasks. Some days you will think you have accepted the loss and then a photo in a shoebox will drop you back to square one. That is not regression. That is grief.

You are not failing at grief. You are doing it exactly as it works: not in a straight line, not on a schedule, and not according to anyone else's timeline.

What Grief Needs

Grief needs time, space, and witness. It needs someone to sit with it without trying to fix it. It needs permission to be ugly,

slow, and non-linear.

Give yourself:

• Permission to grieve the relationship you thought you had — regardless of whether you stay in it.

• Permission to feel loss alongside love. You can still love this person and grieve what their choices took from you. These are not contradictory.

• Permission to grieve on your own timeline. Grief does not have a standard duration. It takes as long as it takes.

• A space — in therapy, with a trusted person, in your own private writing — where the grief can come out fully without you having to manage its impact on the other people in your life.

If the grief is accompanied by an inability to function — to eat, sleep, work, care for yourself — please reach out to a mental health professional. Grief and clinical depression can look similar but require different support.

CHAPTER NINE

The Third Party

The Scene

Sarah has not looked her up. She made this decision on purpose, on the night she found out, and she has kept it for six months.

She does not know what this woman looks like. She does not know where she works. She knows only her first name, which she learned from the messages she was not supposed to see, and which she cannot now unsee.

Today she is driving to the grocery store and a woman turns the corner ahead of her on foot. Dark hair. Sarah's approximate age. Nothing distinguishing.

And Sarah's hands go white on the steering wheel.

It is not her. Sarah knows this. It could be anyone. It is no one.

But for four seconds it is her — the specific her, the one who knew about Sarah and chose anyway, the one who looked at everything Sarah did not know she had and decided it was something to take. And Sarah is flooded.

She pulls over. She sits. She breathes.

She has not looked her up.

She will not.

But she is everywhere.

Why You Cannot Stop Thinking About Her

The third party lives rent-free in the minds of most betrayed partners. This is not a character flaw. It is not obsession in the clinical sense. It is the natural result of a specific kind of injustice that the brain cannot resolve.

Your partner was a person you chose. They hurt you. That is one kind of wound — the main wound.

But the third party is a stranger who participated in your harm without your knowledge or consent. A stranger who knew about you — or chose not to know — and proceeded anyway. This is an injury without an accessible address. There is no conversation to have. No apology to receive. No relationship to repair. Just a person-shaped hole in your reality that your brain keeps trying to fill with information.

This is why the compulsive thinking happens. The brain's threat-detection system identified a real threat, and it wants data. It wants to categorize the danger. It wants to understand: who is this person, why did this happen, what does she have that I don't have, what does she look like, is she still in contact with my partner?

These are the questions your brain is asking. They are not crazy questions. They are not evidence that you are 'obsessed.' They are the sound of your nervous system trying to make sense of a threat it cannot fully process.

The Comparison Trap

One of the most painful versions of third-party thinking is the comparison spiral: the relentless comparison of yourself to this other person. Am I prettier? Am I thinner? Am I more interesting, more available, more something? What did she give him that I didn't?

This spiral will eat you alive if you let it. Here is why it is built on a false premise:

The affair was not a referendum on you. It was not a comparative evaluation of two women in which the better woman won. It was a series of choices your partner made — driven by their own internal landscape, their patterns, their avoidance, their character — that had nothing to do with your adequacy.

The third party was not chosen over you. Nothing was evaluated and found wanting. Your partner made choices that had everything to do with who they are and very little to do with who you are.

The comparison spiral also has a secondary injury built into it: it centers the third party in your self-concept. It makes her the measuring stick by which you evaluate your own worth. She does not deserve that position. She did not earn it. And you deserve better than to locate your value in relation to someone who participated in your harm.

To Look or Not to Look

Many betrayed partners face a specific decision: do I look her up? Do I find her social media, her picture, her professional profile, her life?

Some people do. Some find it provides a kind of grim closure: now I know. Some find it makes the intrusive thoughts worse.

There is no universal right answer. But there are a few questions worth asking yourself before you go down that road:

• What am I hoping to feel on the other side of this? If the answer is 'better,' that outcome is unlikely.

• What information would actually help me? Very often, the specific details you imagine wanting to know will not give you what you're actually looking for (which is usually a sense of safety and understanding, not facts).

• Am I regulated enough to handle what I find? If you are in an acute trigger state, this is not the time to go looking for something that will intensify the flood.

If you choose to look, go in with realistic expectations. The information will not resolve the underlying wound. It will not answer the questions your body is really asking (Why wasn't I enough? Why did he choose this?). It may give you facts but not peace.

What to Do With the Intrusive Thoughts

Whether you look or not, the intrusive thinking will continue for some time. Here is how to work with it rather than against it:

1. Acknowledge the thought without acting on it. 'I am having a thought about her right now. That thought is there because my brain is trying to process a threat. I do not have to follow this thread.'

2. Name what the thought is really about. Underneath the intrusive thinking about her is usually a core fear: I am not enough. I was not loved. My partner preferred someone else. These fears — not the facts about the third party — are what need addressing.

3. Bring the fear, not the facts, to therapy. Your therapist cannot help you understand her. Your therapist can help you understand the fear underneath the obsession with her. That is the work that will actually move.

4. Set a containment practice. Some people find it helpful to designate a specific time and duration for 'thinking about this' — fifteen minutes in the morning, in a journal. Outside of that window, when the thought comes, you redirect: 'I'll think about this in my designated time.' This sounds too simple. It works.

5. Ask your partner to be transparent about ongoing contact. One major driver of ongoing third-party obsession is uncertainty: is he still in contact with her? If your partner can provide clear, verifiable evidence of no ongoing contact, this often reduces the intensity of the intrusive thinking. This is a reasonable request.

CHAPTER TEN

When the People Around You Get It Wrong

The Scene

It is a Friday evening, six weeks after D-Day, and Sarah is having dinner with her two closest friends.

She told them. She needed people to know. She needed somewhere to put this besides her own chest.

What she did not anticipate was that they would have opinions.

Friend A: (leaning forward) I just think you need to leave. Like, I'm sorry, but you leave. That's what you do.

Friend B: (carefully) I mean — you guys have been together for so long. Maybe this was just... a mistake. Everyone makes mistakes.

Sarah looks at the wine in her glass.

She does not want to leave. She does not want to minimize it as a mistake.

She wanted someone to say: This is devastating, and you are going to be okay, and I am here.

Neither of them said that.

She will spend the drive home more alone than when she arrived.

Why Well-Meaning People Usually Get This Wrong

The people who love you will respond to your betrayal from their own emotional systems. They will be frightened for you, furious on your behalf, or flooded with their own unresolved fears about relationships. Their response will reflect all of that — and very little of what you actually need.

Friend A sees betrayal as an emergency exit. For her, love is not worth this kind of pain, and the only dignified response is to leave. She is responding from her own values and possibly her own wounds.

Friend B is uncomfortable with the complexity. She wants this to be resolvable, wants you to be okay, wants the story to have a clean redemption arc. She is minimizing because ambiguity is hard to sit with.

Neither of them is responding to you specifically. Both of them are responding to their own discomfort with your situation.

People who love you will often give you advice that is really about what they would do — or what they need to believe — rather than what is right for you.

The Problem with Taking Sides

When you share your situation with people close to you, you are often unwittingly enlisting them in your pain. They become allies in your corner — which feels supportive in the short term.

The problem is that people who are enlisted as allies cannot easily un-enlist. If you and your partner reconcile, your allies may not reconcile along with you. You may find yourself defending your partner to the very people you asked to support you. You may find that your friendships now contain a painful history that you wish you could erase but cannot.

This is not a reason to stay silent and carry this alone. But it is a reason to be thoughtful about who you tell, how much you tell them, and what you are asking them to hold.

What You Actually Need

What most betrayed partners need from their support network is simpler and harder than advice:

• Presence without prescription. Someone who will sit with you in the pain without telling you what to do about it.

• Validation without escalation. Someone who can say 'that is not okay' without taking it to a place that makes your situation feel more catastrophic than it already is.

• Confidentiality. Someone who will not tell other people, who will not 'check in' on your situation with mutual friends, who can be trusted with something that is not theirs to share.

• Following your lead. Someone who listens to where you are — not where they think you should be.

Very few people are naturally equipped to provide all of this. This is not a criticism of the people who love you. It is a reality of human emotional capacity. Most people will project, advise, minimize, or catastrophize — because that is what people do when they care and feel helpless.

How to Ask for What You Need

You can actually tell people what kind of support you need. This feels awkward, but it works. Here are some examples:

"I need to talk about this, but I don't need advice. I just need you to listen."

"I'm not ready to make any decisions yet. I just need someone to sit with me in the uncertainty."

"I need you to not tell me what you would do. I need you to ask me what I want."

"I need to know this stays between us. I need to trust that completely before I share."

People who love you will usually try to give you what you ask for when you are this specific. The problem is we rarely ask this specifically. We share our pain and then hope the other person

will intuit what we need. They cannot. Tell them.

When to Protect the Information

Some information about your situation should be protected from some people. Ask yourself:

• Will knowing this change how this person treats my partner permanently? If you reconcile, can you live with the fallout of this person's knowledge?

• Is this person able to hold this information without acting on it? (Some people will confront your partner. Some will tell others. Some will make this about themselves.)

• Am I sharing this because it will help me, or because I am flooded and need an outlet that isn't actually useful?

Your parents, your children, your in-laws, your shared friends — all of these categories require specific thought before sharing. The information cannot be unshared once it is out.

Professional support — individual therapy, couples counseling, support groups for betrayed partners — exists precisely for this reason. These are spaces where you can be fully honest without the relational consequences that come from sharing with people who are also in your life in other ways.

Use those spaces. They are there because human beings are not equipped to bear witness to each other's most devastating experiences without becoming part of the story themselves. Professionals are trained to witness without becoming part of the narrative.

CHAPTER ELEVEN

If There Are Children

The Scene

It is Saturday morning, seven weeks in. Sarah and Mike are sitting in the kitchen with their coffee — a distance of approximately four feet and approximately six hundred miles between them.

Their daughter, Maggie, is seven. She comes in with bedhead and a stuffed rabbit and climbs onto the barstool next to her mother.

Maggie: Why does Daddy sleep in the other room?

The air changes. Mike's hands go still around his mug. Sarah looks at the table.

Sarah: (after a moment) He's been snoring a lot, baby. It was keeping Mommy awake.

Maggie considers this.

Maggie: Is he sick?

Sarah: No, sweetheart. He's fine.

Maggie: (to Mike, matter-of-factly) You should see a doctor about the snoring.

Mike: (quietly) You're right. I should.

Maggie slides off the stool and goes to watch cartoons. The kitchen goes quiet.

Sarah and Mike do not look at each other.

This is the part of the books they hadn't read.

What Children Know and Don't Know

Children are acutely sensitive to emotional undercurrents in the home. They may not be able to name what is wrong, but they know something is. They register the tension in their parents' bodies, the change in household rhythms, the altered quality of touch and conversation.

What they do not know — because they are children, and the world they are building is a world in which they are central — is that what is happening is not about them. Children are developmentally egocentric. When the atmosphere changes, their first instinct is often: did I cause this? Is something wrong with me? Are my parents going to split up? Am I going to lose something?

These fears do not require articulation to be present. They show up as behavioral changes: increased clinginess, sleep disturbances, regressive behavior, irritability, withdrawal. Your child may not say 'I am scared.' They may wet the bed, or refuse to go to school, or become suddenly demanding, or go very quiet.

Your job is to provide reassurance without disclosure. This is a narrow and difficult path.

What to Tell Children (by Developmental Stage)

Children Under 6

Very young children do not need explanations. They need reassurance and stability. The snoring explanation, or 'Daddy isn't sleeping well,' or 'Mommy and Daddy are working on some grown-up stuff' is generally sufficient — as long as it is delivered calmly and followed by normal daily routine.

What young children need most: physical affection from both parents, maintained routine, and visible evidence that both parents are present and safe.

Children Ages 6 to 12

Older children will ask more direct questions and will be less satisfied with deflection. They need simple, non-detailed truth that preserves their security:

"Mommy and Daddy are going through a hard time right now. We are working on it. It is not your fault. We both love you exactly as much as we always have, and that is not going to change."

They may ask follow-up questions. You do not have to answer all of them. 'That's a grown-up thing' is an acceptable response to questions you cannot answer in age-appropriate terms. What matters is that the core message — you are safe, this is not your fault, we love you — is delivered clearly and returned to whenever needed.

Teenagers

Adolescents are perceptive in ways that can be both useful and painful. They may already know more than you think. They may have picked up on the change in household atmosphere and drawn their own conclusions.

With teenagers, a slightly more honest framing can work: 'Your dad and I are going through something hard right now. We're working on it. I don't want to give you details that aren't yours to carry, but I want you to know it's real and we're taking it seriously.'

What teenagers need most: to not be enlisted as confidants, advisors, or allies. The moment you share details with a teenager — even fragments, even 'I just need someone to talk to' — you have handed them a burden they cannot put down and that will shape their relationship with both their parents for years.

What Not to Do

• Do not use your child as a messenger. Do not ask them to tell the other parent something you cannot bring yourself to say directly.

• Do not speak negatively about your partner in front of your child — even in fragments, even in tone of voice, even in eye rolls. Children absorb all of it. It damages their relationship with both parents.

• Do not allow your child to become your emotional support system. This is parentification, and it causes real harm. Your child needs you to be the parent right now, not the other way around.

• Do not reassure your child that 'nothing is wrong' when something clearly is. They will not believe you, and it will teach them that their perceptions are not trustworthy.

• Do not make promises about the future that you cannot keep ('We are not getting divorced') if you genuinely do not know what the future holds.

Getting Support for Your Child

If your child shows significant behavioral changes, or if the disruption in the household is prolonged, consider consulting with a child therapist. You do not have to explain everything to the therapist — 'our household is going through a stressful period' is sufficient for them to support your child without needing the details.

Child therapists are trained to work with children who are experiencing disruption without knowing its source. They can provide your child with a space to process their feelings and a toolkit for managing the uncertainty — while you work on yours.

Your children need both of their parents to stay parents right now. The best thing you can do for them is to get enough support for yourself that you can remain present and regulated in their presence.

CHAPTER TWELVE

Rebuilding Intimacy on Your Terms

The Scene

It is a Tuesday night in month seven.

They have been sleeping in the same bed again for three weeks. This was Sarah's decision — she made it on a quiet Thursday evening when she realized that lying alone in the dark was no longer less frightening than lying next to him in the dark.

Tonight Mike reaches for her hand in the bed. Just her hand. Nothing else.

She lets him hold it.

She is aware of the quality of the silence between them — not the hostile silence of the early months, or the exhausted silence of the middle months. Something different. Something that contains, somewhere inside it, the faint outline of what they used to be.

She does not move toward him. She does not pull away.

She holds his hand.

That is enough, for tonight.

That is more than it has been in a long time.

The Body's Complicated Relationship with Intimacy After Betrayal

Sexual betrayal creates a specific kind of harm to the body. Your body is not simply where your emotions live. It is where your trauma lives. And intimacy — physical closeness, touch, sexual connection — asks your body to be vulnerable in the same way it was vulnerable when it was hurt.

This is why rebuilding physical intimacy after betrayal is not just an emotional project. It is a physiological one. Your nervous system has associated physical closeness with the risk of harm. The same body that used to reach toward your partner now hesitates, or flinches, or goes flat — not because you don't want connection, but because your body has learned that connection is dangerous.

This is a learned response, not a permanent state. It can change. But it changes at the pace of your nervous system, not at the pace of your desire to be 'over it.'

The Difference Between Safety and Desire

Before desire can return, safety must be established. Not performed safety — real safety. The kind that is built through consistent, trustworthy, patient behavior over time.

Many couples make the mistake of attempting to 'use' physical reconnection to accelerate emotional repair — to short-circuit the harder work of rebuilding trust through behavior. This can feel healing in the moment. In the medium term, it often leaves the betrayed partner feeling used, confused, or more damaged than before.

Physical intimacy that happens before emotional safety is established tends to be experienced by the betrayed partner as pressure — even when the unfaithful partner has the best intentions. The body knows the difference between safe closeness and premature closeness, and it will signal accordingly.

You do not owe your partner physical intimacy on any timeline. Not because you are punishing them. But because your

nervous system has a vote, and it will not be overridden by intention or effort or desire.

Reclaiming Your Own Relationship With Your Body

One of the less-discussed impacts of sexual betrayal is the way it disrupts your relationship with your own body. Your body has been implicated in the betrayal — either as something that was somehow 'insufficient,' or as something that was part of the intimate world that was shared outside the relationship without your consent.

You may find that you feel disconnected from your body. Or that you look at yourself differently. Or that your own pleasure feels complicated in ways it didn't before. These responses are common. They are also, with time and support, workable.

Part of the work of rebuilding intimacy is rebuilding your relationship with your own body independent of your relationship with your partner. This might involve:

• Individual therapy specifically addressing body image and sexuality in the context of betrayal trauma.

• Physical practices that reconnect you with your body on your own terms — exercise, dance, yoga, massage, anything that involves your physical self without the context of the relationship.

• Permission to acknowledge and attend to your own pleasure, separate from partnered intimacy.

• Patience with the ways your sexuality may have shifted, and curiosity about what it wants now — rather than pressure to return to who you were before.

How Rebuilding Actually Goes

Rebuilding physical intimacy after betrayal rarely follows the pattern that couples expect. It is not a linear progression from no touch to more touch to eventual 'normal.' It is jagged, non-linear, and requires ongoing communication.

What typically helps:

• Non-sexual touch first. Hand-holding, a hand on the shoulder, a brief hug — touch that does not carry the weight of sexual expectation. Let this become comfortable before moving to anything more.

• The betrayed partner setting the pace. The unfaithful partner should follow the lead of the betrayed partner in all physical reconnection. Any attempt to accelerate, suggest, or pressure — even subtly — will set back the process significantly.

• Explicit conversation about boundaries and readiness. Not in the moment, but in a calm, non-intimate setting: 'I want to talk about where we are with physical closeness. Here is where I am. What do you need from me, and what do I need from you?'

• Permission to stop. Any time. Without explanation. The betrayed partner needs to know — and believe — that they can stop any physical interaction at any point without creating conflict or resentment.

• Normalizing the non-linear. There will be nights when it feels possible and nights when it does not. Both are okay. The goal is not a consistent upward trajectory but a genuine process of reconnection that respects where both people actually are.

When Intimacy Cannot Come Back

For some partners, sexual intimacy does not come back. Not because of insufficient effort, not because the love is gone, not because the relationship could not be repaired in other ways — but because the body cannot reconcile the specific injury of sexual betrayal with the vulnerability that sexual intimacy requires.

This is a legitimate outcome. It does not represent failure. It may represent a reality that requires a different kind of conversation about the relationship's future — but it is not evidence that you are broken, or that you did not try hard

enough, or that forgiveness wasn't sufficient.

If you find yourself in this situation, please work with a therapist who specializes in sexual trauma and intimate relationships. There is nuanced, specialized support available for exactly this territory.

Your body does not have to forgive on any timeline. What it needs is safety, patience, and the knowledge that its signals will be respected. When it has those things, it often surprises you.

About the Companion Book

A Note on Professional Support

Resources for Support

About the Companion Book

You may have noticed that this book references its companion volume, The Repair Work: For the Unfaithful Partner, throughout these pages. This is not an accident. It is not a sales tactic. It is a clinical intervention.

Why you need to know about the companion book:

This book tells you what happened to you. It validates your experience. It gives you tools for regulation, boundaries, and decision-making.

But it cannot tell you if your partner is actually doing the work. That is what the companion book is for.

The Repair Work: For the Unfaithful Partner is written directly to the person who hurt you. It does not soften the truth. It does not manage their feelings. It tells them what accountability actually looks like — and what it does not.

The companion volume is not a suggestion. It is a boundary.

When you hand your partner this book, you are saying: This is what I need from you. Read it. Sit with it. Do the work it asks you to do. When you are done, we can talk. Not before.

Watch what they do:

- Do they read it?
- Do they dismiss it?
- Do they rush through it and ask you to “fix” them?
- Do they argue with it?

• Do they use it as a weapon to say "I've done everything the book says"?

• Do they sit in the discomfort or run from it?

• Do they come to you changed — not defensive, not performing, but genuinely different?

Their response to this book is your answer. Not their words. Their actions.

The two books work together because the two people in the room need different things. The betrayed partner needs validation, tools, and permission to heal on their own timeline. The unfaithful partner needs accountability, confrontation, and a clear standard for what change actually looks like.

A Note on Professional Support

This book was written to be a companion — not a replacement — for professional mental health support. Betrayal trauma is a clinical-level injury. The physiology documented in these pages, the nervous system disruption, the intrusive memories, the hypervigilance and collapsed trust: these deserve more than a book can offer.

If you are not currently working with a therapist, please consider finding one — ideally someone trained in trauma-informed care, emotionally focused therapy (EFT), or couples therapy specializing in infidelity recovery.

A Note on Safety

This book assumes that you are in a relationship where you can safely set boundaries, express your needs, and make choices about your future without fear of retaliation or harm. If this is not true for you — if your partner has ever been physically violent, threatened you, controlled your access to resources, or isolated you from support — please prioritize your safety above everything else.

The work of repair cannot begin until the abuse ends. If you are experiencing abuse, please reach out to the National Domestic Violence Hotline at 1-800-799-7233 or text START to 88788.

If you are experiencing suicidal thoughts, a mental health crisis, or thoughts of harming yourself or others, please reach out

immediately. The 988 Suicide and Crisis Lifeline is available 24/7 by calling or texting 988.

Resources for Support

The following resources may be helpful as you navigate healing from betrayal trauma. This list is a starting point, not an endorsement.

Crisis and Immediate Support

988 Suicide and Crisis Lifeline — call or text 988, available 24/7.

Crisis Text Line — text HOME to 741741, free, confidential, 24/7.

National Domestic Violence Hotline — 1-800-799-7233 or text START to 88788.

Finding a Therapist

Psychology Today Therapist Directory — psychologytoday.com/us/therapists. Filter for "infidelity," "betrayal trauma," or "emotionally focused therapy."

ICEEFT (International Centre for Excellence in EFT) — iceeft.com. Directory of certified EFT therapists.

NBCC Counselor Find — counselorfind.com.

Betrayal Trauma Specific

BTR.org — Betrayal Trauma Recovery. Support groups, podcasts, and coaching. Free resources available.

AASAT — sexhelp.com. Directory for finding certified sex addiction therapists.

Books That May Help

Bessel van der Kolk, The Body Keeps the Score (2014)

Shirley Glass, Not “Just Friends” (2003)

Emily Nagoski, Come as You Are (2015)

Peter Levine, Waking the Tiger (1997)

Sue Johnson, Hold Me Tight (2008)

Worksheet Appendix

How to Use These Worksheets

Every worksheet in this appendix includes an Environment Considerations box at the top. Please read this section before beginning any exercise. These considerations are not optional — they are part of the clinical design of the worksheet.

Betrayal trauma is a physiological event. Your nervous system does not distinguish between a worksheet prompt and an actual threat. The wrong environment, wrong timing, or wrong emotional state can cause a worksheet exercise to become retraumatizing rather than healing. The environment considerations exist to prevent that.

What "Window of Tolerance" Means

Your window of tolerance is the zone of arousal in which your nervous system can process difficult material without going into survival mode.

• 0–3: Very calm. Light reflection worksheets are fine.

• 4–5: Good zone for most worksheet work. Proceed with full awareness.

• 6–7: Proceed cautiously, or only with a therapist present.

8–10: STOP. Do not begin any worksheet. Use grounding tools first. Contact your therapist.

Standard Grounding Toolkit

• 5-4-3-2-1: Name 5 things you can see, 4 you can touch, 3 you can hear, 2 you can smell, 1 you can taste.

• Vagus nerve breath: In for 4 counts, hold 4, out for 8. Repeat 3–5 times.

• Physical anchor: Press feet into floor, hands on knees, feel the weight of your own body.

• Verbal anchor: "I am being triggered. I am not in danger. I am in the present. This will pass."

Your Support Contacts

My therapist / counselor:

My crisis contact (person I trust):

Crisis line I can call:

National Crisis Line: 988 Suicide & Crisis Lifeline — call or text 988

Worksheet 1

My Trigger Map

Purpose: Identify and name your personal triggers before they ambush you, so your nervous system has a map instead of a minefield.

Part 1: Known Triggers

Sensory triggers (songs, smells, sounds, specific locations):

Situational triggers (specific scenarios, times, contexts):

Relational triggers (things your partner does, says, or fails to do):

Date/time-based triggers (anniversaries, times of day, seasons):

Part 2: What Happens in My Body

My first body sensation is usually:

What happens to my breathing:

What happens to my heart rate:

Where I feel tension in my body:

Signs that I am approaching my limit (before I flood):

Part 3: My Automatic Responses

I tend to (circle all that apply): Withdraw and go quiet | Cry | Get angry | Leave the room | Shut down | Seek reassurance | Ask questions | Go over it again mentally | Reach for my phone

Other automatic responses I notice:

What I most need when triggered:

Part 4: My Regulation Toolkit

My most effective grounding technique:

The breathing approach that helps me most:

Movement that helps me discharge adrenaline:

My verbal anchor (what I say to myself):

The environment I need to regulate (outside, my room, car, etc.):

Part 5: What I Need From My Partner

When I am triggered, what helps most is:

When I am triggered, what makes it worse is:

The one thing I most need you to say is:

The one thing I most need you NOT to say is:

How I will let you know I'm triggered, even if I can't speak:

Worksheet 2
Window of Tolerance Daily Check-In

Purpose: A brief daily self-monitoring tool to help you track your nervous system state and make informed decisions about what you can handle today.

Date: _____

Step 1: Body Scan

Physical sensations I notice right now:

Where I feel tension or tightness:

My breathing is (circle one): Shallow / Normal / Deep / Tight / I'm not sure

Step 2: Distress Level

My current distress level: [1 2 3 4 5 6 7 8 9 10]

My window of tolerance today (circle one): OPEN (4–6) · NARROW (6–7) · CLOSED (8+: stop, ground, reach out)

Step 3: Today's Capacity

What I feel capable of handling in my relationship today:

What I need to protect myself from today:

One thing I can do today to support my own healing:

Step 4: End-of-Day Reflection

My distress level at end of day: [1 2 3 4 5 6 7 8 9 10]

What triggered me today (if anything):

How I regulated:

One thing I'm proud of myself for today:

--- Reproduce this page for daily use ---

Worksheet 3

My Boundaries and Safety Plan

Purpose: Articulate the non-negotiable conditions you need to begin healing — and establish a clear plan for what you will do if those conditions are violated.

Part 1: My Non-Negotiable Safety Conditions

Around transparency (phone, location, communication):

Around honesty (what I need them to tell me, how quickly, about what):

Around patience and pacing (what I need them NOT to do regarding my timeline):

Around their individual work (therapy, accountability, understanding why):

Around intimacy (what I need before I'm willing to rebuild physical closeness):

Other conditions not listed above:

Part 2: What Holding These Boundaries Looks Like

If a boundary is crossed, my first response will be:

The boundary I am most afraid to hold is:

The story I tell myself when I consider lowering a boundary:

What I want to tell that story:

Part 3: My Response Protocol

If _____ happens, I will:

▯ Tell my partner directly and clearly what happened and what I need now

▯ Contact my therapist to process this before responding further

▯ Take space to regulate before having a conversation

▯ Revisit whether I can continue in this process

▯ Other:

Part 4: My Support Network

My therapist / counselor:

A trusted friend I can be honest with:

A family member who supports me (if applicable):

Crisis line / resource:

Part 5: My Commitment to Myself

I commit to:

Worksheet 4

Healing Stage Inventory

Purpose: Track where you are in the healing process over time — not to measure progress against a standard, but to understand your own experience.

Date: _____ Month _____ of my healing process

Part 1: Where I Am Right Now

Stages (from Chapter 5): The Shattering / The Flood / The Wrestling / The Grieving / The Rebuilding. Stages overlap and repeat.

The stage(s) that best describe where I am this month:

What this stage feels like from the inside right now:

Part 2: What Is Shifting

Trigger frequency (this month vs. last): Much more frequent — to — Much less frequent [1 2 3 4 5 6 7 8 9 10]

Trigger intensity: More intense — to — Less intense [1 2 3 4 5 6 7 8 9 10]

Recovery time: Takes longer — to — Recovering faster [1 2 3 4 5 6 7 8 9 10]

Safety in my own body: Still unsafe — to — Feeling safer [1 2 3 4 5 6 7 8 9 10]

Something that is genuinely easier than it was last month:

Something that is still hard in the same way:

Something that surprised me about my healing this month:

Part 3: Healing Milestones

- ▯ Went a full day without thinking about the affair
- ▯ Had a trigger and recovered within an hour
- ▯ Laughed — really laughed — without feeling guilty
- ▯ Made a decision from clarity rather than fear
- ▯ Had a moment of genuine connection with my partner
- ▯ Chose myself — my needs, my wellbeing — without apology
- ▯ Slept through the night
- ▯ Other:

Part 4: What I Need This Month

From my therapist:

From my partner:

From myself:

One thing I want to focus on for my own healing in the coming month:

Next inventory scheduled: _____

Worksheet 5

Stay or Leave Clarity Worksheet

Purpose: A structured framework for approaching the most important decision you may face — not to rush you toward an answer, but to help you access your own knowing.

IMPORTANT: This worksheet is for you alone. It does not produce a binding decision. It is a thinking tool.

Part 1: Body Check First

When I imagine staying in this relationship, my body feels:

When I imagine leaving, my body feels:

Which response feels like fear, and which feels like truth?

Part 2: The Five Questions From Chapter 6

1. Is my partner truly accountable? (Not just sorry — actually accountable.)

2. Am I safe with my partner? (Emotionally safe — able to be vulnerable without fearing re-injury.)

3. Is my partner genuinely patient? (Not just saying they are — actually demonstrating it.)

4. Is my partner transparent? (Nothing to hide — and proving it consistently.)

5. Am I staying out of love — or out of fear?

Part 3: What I Know

The truth I keep avoiding about staying is:

The truth I keep avoiding about leaving is:

What I most need right now — regardless of what I decide — is:

If I had no fear about the consequences, my gut answer would be:

Part 4: The In-Between

What I need to see or experience before I can answer:

The timeline I am giving myself to continue observing:

DO NOT share with partner until discussed with therapist

Worksheet 6

Safety Assessment: Is This Book Right for You?

Purpose: Before investing significant time and emotional energy in the work this book describes, assess whether your situation is safe enough for repair work.

Part 1: Physical Safety

Answer honestly. If you answer "yes" to any of them, this book is not for you.

Has my partner ever hit, pushed, slapped, choked, or physically hurt me? ▫ Yes ▫ No

Has my partner ever threatened to hurt me? ▫ Yes ▫ No

Has my partner ever threatened to hurt themselves if I leave? ▫ Yes ▫ No

Has my partner ever threatened to hurt someone I love? ▫ Yes ▫ No

Am I afraid of what my partner might do if I try to leave? ▫ Yes ▫ No

Am I afraid of what my partner might do if I set a boundary? ▫ Yes ▫ No

If you answered "yes" to any of these questions, please stop. Contact the National Domestic Violence Hotline at 1-800-799-7233 or text START to 88788. Your physical safety is the priority.

Part 2: Coercive Control

Does my partner control my access to money or financial resources? ▫ Yes ▫ No

Does my partner monitor my phone, email, or social media? ▫ Yes ▫ No

Has my partner isolated me from friends or family? ▫ Yes ▫ No

Does my partner use the children to control me? ▫ Yes ▫ No

Does my partner blame me for things that are not my fault? ▫ Yes ▫ No

Does my partner make me feel like I'm "crazy" or "too sensitive"? ▫ Yes ▫ No

Do I feel like I have to walk on eggshells to keep my partner calm? ▫ Yes ▫ No

---

If you answered "yes" to three or more of these questions, this book may not be safe for you. Please consider contacting a domestic violence advocate for support.

Part 3: Your Partner's Readiness

Has my partner taken full responsibility for the affair — without excuses or minimizing? ▫ Yes ▫ No

Is my partner willing to go to individual therapy? ▫ Yes ▫ No

Is my partner patient with my healing — not rushing me, not pressuring me? ▫ Yes ▫ No

Is my partner transparent — phone accessible, honest about where they are? ▫ Yes ▫ No

Is my partner willing to read the companion volume? ▫ Yes ▫ No

If you answered “no” to three or more of these questions, this book cannot help you. Your partner is not ready to do the work. You cannot do the work for them.

Part 4: Your Readiness

Am I ready to do the hard work of healing — even if it means feeling worse before I feel better? ☐ Yes ☐ No

Am I able to set and hold boundaries? ☐ Yes ☐ No

Am I able to seek and accept professional support? ☐ Yes ☐ No

Am I ready to make decisions based on what is best for me? ☐ Yes ☐ No

What This Means for You

If you answered “yes” to any Part 1 questions: STOP. Contact the National Domestic Violence Hotline at 1-800-799-7233. You deserve to be safe.

If you answered “no” to most Part 3 questions: This book cannot help you. Your partner is not ready.

If most of your answers are in the safe zones: This book is for you. Read on. Do the work. Heal at your own pace.

Date completed: _____ Shared with therapist: ___ (date)

References

The following sources informed the clinical frameworks, statistical data, and therapeutic concepts presented in this book.

Infidelity, Betrayal, and Post-Traumatic Stress

Allen, E. S., Atkins, D. C., Baucom, D. H., Snyder, D. K., Gordon, K. C., & Glass, S. P. (2005). Intrapersonal, interpersonal, and contextual factors in engaging in and responding to extramarital involvement. Clinical Psychology: Science and Practice, 12(2), 101–130.

Cano, A., & O'Leary, K. D. (2000). Infidelity and separations precipitate major depressive episodes and symptoms of nonspecific depression and anxiety. Journal of Consulting and Clinical Psychology, 68(5), 774–781.

Ortman, D. C. (2005). Post-infidelity stress disorder. Journal of Psychosocial Nursing and Mental Health Services, 43(10), 46–54.

Schneider, J. P., Irons, R. R., & Corley, M. D. (1999). Disclosure of extramarital sexual activities by persons with addictive disorders. Sexual Addiction & Compulsivity, 6(1), 5–27.

Spring, J. A. (2012). After the affair: Healing the pain and rebuilding trust when a partner has been unfaithful (3rd ed.). William Morrow.

Steffens, B. A., & Rennie, R. L. (2006). The traumatic nature of disclosure for wives of sexual addicts. Sexual Addiction & Compulsivity, 13(2–3), 247–267.

Trauma, Nervous System, and Neuroscience

Herman, J. L. (1992). Trauma and recovery: The aftermath of violence—from domestic abuse to political terror. Basic Books.

LeDoux, J. E. (1996). The emotional brain: The mysterious underpinnings of emotional life. Simon & Schuster.

Levine, P. A. (1997). Waking the tiger: Healing trauma. North Atlantic Books.

Porges, S. W. (2011). The polyvagal theory: Neurophysiological foundations of emotions, attachment, communication, and self-regulation. W. W. Norton & Company.

van der Kolk, B. A. (2014). The body keeps the score: Brain, mind, and body in the healing of trauma. Viking.

Siegel, D. J. (2010). The mindful therapist: A clinician's guide to mindsight and neural integration. W. W. Norton & Company.

Grief, Loss, and Recovery

Boss, P. (1999). Ambiguous loss: Learning to live with unresolved grief. Harvard University Press.

Boss, P. (2006). Loss, trauma, and resilience: Therapeutic work with ambiguous loss. W. W. Norton & Company.

Worden, J. W. (2009). Grief counseling and grief therapy: A handbook for the mental health practitioner (4th ed.). Springer.

Couples Therapy and Relationship Repair

Baucom, D. H., Snyder, D. K., & Gordon, K. C. (2009). Helping couples get past the affair: A clinician's guide. Guilford Press.

Glass, S. P. (2003). Not "just friends": Rebuilding trust and recovering your sanity after infidelity. Free Press.

Gordon, K. C., Baucom, D. H., & Snyder, D. K. (2004). An integrative intervention for promoting recovery from extramarital affairs. Journal of Marital and Family Therapy, 30(2), 213–231.

Johnson, S. M. (2004). The practice of emotionally focused couple therapy (2nd ed.). Brunner-Routledge.

Johnson, S. M. (2008). Hold me tight: Seven conversations for a lifetime of love. Little, Brown and Company.

Gottman, J. M., & Silver, N. (2015). The seven principles for making marriage work (rev. ed.). Harmony Books.

Snyder, D. K., Baucom, D. H., & Gordon, K. C. (2007). Getting past the affair: A program to help you cope, heal, and move on. Guilford Press.

Sexuality, Shame, and Accountability

Brown, B. (2012). Daring greatly. Gotham Books.

Nagoski, E. (2015). Come as you are. Simon & Schuster.

Perel, E. (2006). Mating in captivity. HarperCollins.

Perel, E. (2017). The state of affairs: Rethinking infidelity. HarperCollins.

Weiss, R. (2017). Out of the doghouse: A step-by-step relationship-saving guide for men caught cheating. Health Communications.

Domestic Violence and Safety

National Domestic Violence Hotline. (n.d.). About financial abuse and coercive control. Retrieved from https://www.thehotline.org

Acknowledgments

This book would not exist without the clients who trusted me with their most fractured moments. They are not in these pages by name, but they are in every line. What I know about betrayal trauma I learned from them — from watching them survive what felt unsurvivable, and from the privilege of sitting with them in the darkest rooms until the light came back.

To the clinicians who came before — the researchers, the theorists, the practitioners who built the frameworks I have stood on — I am grateful. The work of Sue Johnson, John Gottman, Bessel van der Kolk, Peter Levine, and Brené Brown is woven into the fabric of this text, even where their names do not appear.

To every person reading this book: you did not deserve what happened to you. You are not less-than for struggling to heal from it. Your healing matters — not because of what it means for your relationship, but because of what it means for you.

With gratitude,

Hinesville, Georgia

About the Author

Known to her readers as KekeTheLPC, kejuiana Johnson is a multistate-licensed professional counselor, clinical supervisor, and the founder of GA Integrated Therapeutic Perspectives LLC — an NBCC-approved continuing education provider serving mental health professionals across the country.

She holds active clinical licenses in Georgia, Florida, Idaho, and Texas, and brings to her practice a deep commitment to treating the whole person: nervous system and story, body and belief, the wound and the way through. Her specialized focus includes betrayal trauma, relationship repair, attachment injuries, and the neuroscience of healing.

As the creator of CounselorReady™, a professional development platform for licensed clinicians, and PassReady Prep™, a clinical examination preparation resource, Kejuiana has built a career around the conviction that clarity is a form of care.

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To connect with Kejuiana's professional resources, CE courses, or to inquire about clinical supervision:

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Hinesville, Georgia

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Printed in the United States of America